PathwayAI | Dutchess County Lyme Burden & Action Report

PathwayAI: Making the Hidden Patient Journey Visible—and Actionable

New York · Public evidence and support planning · Report version 1.0

1. OBSERVED PUBLIC EVIDENCE

Reported Lyme cases: 1,060 · 2024 · county FIPS 36027.

Reported rate: 355.5 per 100,000 residents. New York State: 109.8.

Surveillance describes reported cases by residence, not all infections or where exposure occurred.

2. STUDY-BASED ECONOMIC ILLUSTRATION

1,060 reported cases × USD 2,032 study mean = USD 2,153,920 (2016 dollars).

613 participants in societal analysis; reported cases recruited in four high-incidence states during 2014–2016.

Societal: medical, nonmedical and productivity costs; components are already included.

Applying this study mean to reported county cases is an assumption; the result is not measured local spending.

No adjustment for underreporting or local case mix. No causal savings or disability probabilities are inferred.

Hook excluded non-English speakers and nonacute disease; the study does not represent every local household.

3. ADDITIONAL BURDEN TO MEASURE

- Lyme-attributable county disability and duration
- Continuing costs beyond the study follow-up
- Local unmet care and support needs
- Experiences of people excluded or underrepresented in the source study

4. SUPPORT DECISION TO EXPLORE

Care without insurance and medical bills

Planning question: Could existing services meet reported uninsured-care needs, or would an additional funded response be needed?

Possible response: Review health-center sliding-fee access, coverage enrollment help and hospital financial assistance with patients; identify staffing, appointment capacity, costs and a possible funding source before proposing new services.

Potential partners to approach: Patients, health centers, hospital financial-assistance staff, enrollment assisters, county planners and potential funders

These are proposed discussion topics, not findings about local demand or commitments from partners.

5. CHECK FEASIBILITY BEFORE A PILOT

- Confirm local demand with residents and caregivers, including assisted/offline participation.
- Verify current services, eligibility, interpreter access and appointment capacity.
- Agree who is responsible and estimate staff time, program costs and a funding source.
- Define consent, minimal data collection, access, retention and deletion before any collection.

6. AGREE ON FOLLOW-UP

Candidate measures: Care appointments reached; enrollment assistance completed; financial-assistance decisions; remaining bills and unmet needs over a common period

Agree a common reporting period and baseline, record missing answers, measure support received and unresolved needs. Before/after changes alone do not establish causality or savings.

SOURCES AND METHODS

Reported cases, county of residence · 2024

https://www.health.ny.gov/statistics/diseases/communicable/2024/docs/cases.pdf

Previously checked supplied official PDF, page 5.

Reported rates per 100,000 · 2024

https://www.health.ny.gov/statistics/diseases/communicable/2024/docs/rates.pdf

Previously checked supplied official PDF, page 5.

Societal study mean · 2016 dollars; recruitment 2014–2016

https://wwwnc.cdc.gov/eid/article/28/6/21-1335_article

Checked user-supplied public article text: Methods, Results and Discussion. Linked tables not separately retrieved. Live website blocked.

Public evidence and proposed planning actions only. No session stories, personal records or synthetic patient cards are included.

Discussion brief, not a county demand assessment, benefits decision, funded service or adopted county plan.